Psychological Reactance to COVID-19 Mandates and Its Legacy on Public Health

During the height of COVID-19, governments and corporations implemented vaccine mandates to boost immunity and protect the public. While these policies raised immunization numbers, vaccine mandates also triggered intense psychological reactions that still shape public resistance and political momentum against current and future public health mandates.

Behind coercive health legislation is the Psychological Reactance Theory. It was first created by Jack Brehm in 1966. The PRT states that people feel an urge to reassert their individual freedom because they feel that it is being threatened. For example, when an individual feels like an employer is attempting to restrict their freedoms, psychological reactance is triggered and causes anger, anxiety, and other negative cognitions. To get rid of such feelings, individuals feel the need to rebel. In the case of COVID-19, refusing the vaccine or participating in anti-mandate protests.

Although the intent behind vaccine mandates were to encourage public health, the mandates instead suggested opposition. Some of the following sources demonstrate the global psychological mechanisms that backfired:

- In Germany, the Sprengholz et al. study shows that mandates caused Twitter users to post tweets exhibiting negativity, anger, and oppressed language.
    - The negative behavior demonstrates how mandates induced avoidance to COVID-19 vaccines.
- In the United Kingdom, the De Figuerido et al. study illustrates how mandates provoked street protests.
    - The vaccine passports caused a decrease in the inclination among individuals to get vaccinated.
- In the United States, Self-determination studies evaluated the attitudes regarding COVID-19 vaccine mandates.
    - This study showcased how autonomy was identified as the number one indicator for a person to be willing to accept medical intervention.

Overall, these examples exemplify how mandates deepened public skepticism. People felt oppressed which caused policy complaints to turn into general institutional distrust.

Furthermore, it is interesting to note that psychological reactance did not disappear after the pandemic ended. The anger built up from COVID-19 even caused the public to question other already established immunization programs.

Evidence shows that individuals who experienced high levels of reactance concerning COVID-19 mandates demonstrated decreased acceptance of other non-mandated vaccines (such as influenza or varicella). The coercion behind COVID-19 shifted the population”s mindset to question all medical recommendations.

The long-term legacy of this psychological reactance can be seen in today's legislation. The psychological effect behind COVID-19 mandates catalyzed a powerful counter momentum. A strong case that studies this shift can be seen in the United States, where many states enacted bans against COVID-19 and vaccine mandates.

Research published on the Research Gate database evaluated the impact of state bans. Using a geographic discontinuity design, researchers discovered that counties who banned mandates actually saw a 38-43% increase in vaccinations following the ban.

The psychological explanation behind vaccine mandates is simple: Once an individual's perceived threat to freedom is resolved, psychological reactance dissolves. When an individual doesn't feel coerced, they are comfortable enough to accept the vaccine on their own conditions.

In summary, the psychological reactance behind the pandemic proves that public health cannot be separated from human behavior. If policies ignore the human need for freedom, it triggers an opposite reaction. The momentum which keeps anti-mandate legislation alive is fueled by one lesson: protecting individual freedoms is the most effective way to foster public trust.

Citations

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